

Patient Name : Ms.VENKATAMMA G
Age/Gender : 77 Y 11 M 15 D /F
UHID/MR No : APJ1.0031336678
Visit ID : DTPTOPV38584
Ref Doctor : Dr.SELF
IP/OP NO :

Collected : 17/Aug/2026 08:23AM
Received : 17/Aug/2026 01:19PM
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Status : Final Report
Client Name : APOLLO 24X7
Center location : ANNAMAYYA CIRCLE,Tirupathi

DEPARTMENT OF HAEMATOLOGY

APOLLO FULL BODY CHECKUP - PLATINUM WITH SMART REPORT

Test Name	Result	Unit	Bio. Ref. Interval	Method
COMPLETE BLOOD COUNT (CBC) , WHOLE BLOOD EDTA				
HAEMOGLOBIN	10.8	g/dL	12.5-15	Spectrophotometer
PCV	33.80	%	36-46	Electronic pulse & Calculation
RBC COUNT	4.15	Million/cu.mm	3.8-4.8	Electrical Impedance
MCV	81.6	fL	83-101	Calculated
MCH	26	pg	27-32	Calculated
MCHC	31.9	g/dL	31.5-34.5	Calculated
R.D.W	15.8	%	11.6-14	Calculated
TOTAL LEUCOCYTE COUNT (TLC)	7,980	cells/cu.mm	4000-10000	Electrical Impedance
DIFFERENTIAL LEUCOCYTIC COUNT (DLC)				
NEUTROPHILS	66.1	%	40-80	Electrical Impedance
LYMPHOCYTES	24.3	%	20-40	Electrical Impedance
EOSINOPHILS	3	%	1-6	Electrical Impedance
MONOCYTES	5.9	%	2-10	Electrical Impedance
BASOPHILS	0.7	%	0-2	Electrical Impedance
CORRECTED TLC	7,980	Cells/cu.mm		Calculated
ABSOLUTE LEUCOCYTE COUNT				
NEUTROPHILS	5274.78	Cells/cu.mm	2000-7000	Calculated
LYMPHOCYTES	1939.14	Cells/cu.mm	1000-3000	Calculated
EOSINOPHILS	239.4	Cells/cu.mm	20-500	Calculated
MONOCYTES	470.82	Cells/cu.mm	200-1000	Calculated
BASOPHILS	55.86	Cells/cu.mm	0-100	Calculated
Neutrophil lymphocyte ratio (NLR)	2.72		0.78- 3.53	Calculated
PLATELET COUNT	378000	cells/cu.mm	150000-410000	Electrical impedance
MPV	7.9	fL	8.1-13.9	Calculated

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Dr. POOJITHA RAM. V
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SIN No:HA11835177



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DEPARTMENT OF BIOCHEMISTRY

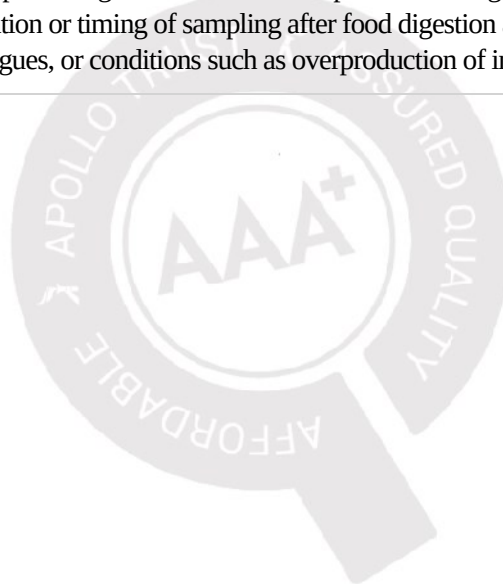
APOLLO FULL BODY CHECKUP - PLATINUM WITH SMART REPORT

Test Name	Result	Unit	Bio. Ref. Interval	Method
GLUCOSE, POST PRANDIAL (PP), 2 HOURS , SODIUM FLUORIDE PLASMA (2 HR)	252	mg/dL	70-140	GOD - POD

Comment:

It is recommended that FBS and PPBS should be interpreted with respect to their Biological reference ranges and not with each other.

Conditions which may lead to lower postprandial glucose levels as compared to fasting glucose levels may be due to reactive hypoglycemia, dietary meal content, duration or timing of sampling after food digestion and absorption, medications such as insulin preparations, sulfonylureas, amylin analogues, or conditions such as overproduction of insulin.



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Poojitha Ram V

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DEPARTMENT OF BIOCHEMISTRY

APOLLO FULL BODY CHECKUP - PLATINUM WITH SMART REPORT

Test Name	Result	Unit	Bio. Ref. Interval	Method
HBA1C (GLYCATED HEMOGLOBIN) , WHOLE BLOOD EDTA				
HBA1C, GLYCATED HEMOGLOBIN	9.4	%		HPLC
ESTIMATED AVERAGE GLUCOSE (eAG)	223	mg/dL		Calculated

Comment:

Reference Range as per American Diabetes Association (ADA) 2023 Guidelines:

Reference Group	HbA1c (%)	HbA1c (mmol/mol)
Non-Diabetic	<5.7	<38.8
Prediabetes	5.7 – 6.4	38.8 – 46.5
Diabetes	≥ 6.5	≥47.5
Diabetics		
Excellent control	6 – 7	42.1 – 53.0
Fair to good control	7 – 8	53.0 – 63.9
Unsatisfactory control	8 – 10	63.9 – 85.8
Poor control	>10	>85.8

Note: HbA1c IFCC (mmol/mol) = (10.93 x HbA1c NGSP (%)) – 23.50

- HbA1C is recommended by American Diabetes Association for Diagnosing Diabetes and monitoring Glycemic Control by American Diabetes Association guidelines 2023.
- Trends in HbA1c values is a better indicator of Glycemic control than a single test.
- Low HbA1c in Non-Diabetic patients are associated with Anemia (Iron Deficiency/Hemolytic), Liver Disorders, Chronic Kidney Disease. Clinical Correlation is advised in interpretation of low Values.
- Falsely low HbA1c (below 4%) may be observed in patients with clinical conditions that shorten erythrocyte life span or decrease mean erythrocyte age. HbA1c may not accurately reflect glycemic control when clinical conditions that affect erythrocyte survival are present.
- In cases of Interference from Haemoglobin variants (HbF >25%, Homozygous Hemoglobinopathies) in HbA1C testing, alternative methods (Fructosamine) estimation is recommended for Glycemic Control. Abnormal Haemoglobin studies (HPLC/Electrophoresis) is recommended for detection of Hemoglobinopathies.

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Dr. POOJITHA RAM. V
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SIN No:BI34067463



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DEPARTMENT OF BIOCHEMISTRY

APOLLO FULL BODY CHECKUP - PLATINUM WITH SMART REPORT

Result Report

PatientID:

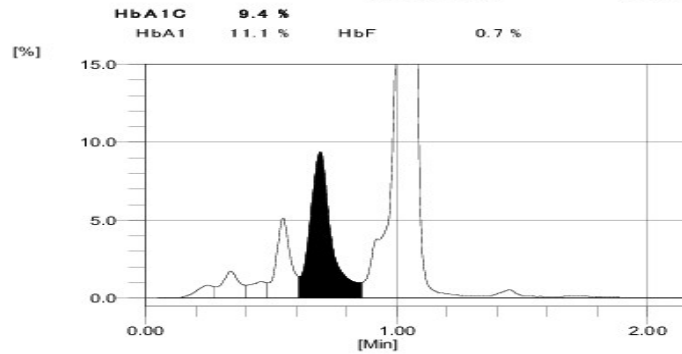
Record Date: 2026-08-17 15:03:13

Barcode: BI34067463

SampleNo: 08170023

CALIB: $Y = 1.1683X + 0.5714$

NAME	%	TIME	AREA
A1A	0.5	0.25	6.03
A1B	1.2	0.34	12.80
F	0.7	0.46	7.85
LA1C+	2.8	0.55	31.52
SA1C	9.4	0.70	84.68
A0	87.8	1.04	971.54
TOTAL AREA			1114.42



2026-08-17 15:26 root

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Poojitha Ram V

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DEPARTMENT OF BIOCHEMISTRY

APOLLO FULL BODY CHECKUP - PLATINUM WITH SMART REPORT

Test Name	Result	Unit	Bio. Ref. Interval	Method
LIPID PROFILE , SERUM				
TOTAL CHOLESTEROL	166	mg/dL	0-200	CHOD-PAP
TRIGLYCERIDES	297	mg/dL	0-149	Enzymatic
HDL CHOLESTEROL	30	mg/dL	30-85	Direct
NON-HDL CHOLESTEROL	136	mg/dL	<130	Calculated
LDL CHOLESTEROL	76.6	mg/dL	<100	Calculated (Friedewald)
VLDL CHOLESTEROL	59.4	mg/dL	<30	Calculated
CHOL / HDL RATIO	5.53		0-4.97	Calculated
ATHEROGENIC INDEX (AIP)	0.640		<0.11	Calculated

Comment:

Reference Interval as per National Cholesterol Education Program (NCEP) Adult Treatment Panel III Report.

Below Table as per Lipid Association of India (LAI) (2023) and Cardiological Society of India (CSI) (2024) Guidelines and Consensus Statements for Dyslipidemia Management:

	Low Risk	Moderate Risk	High Risk	Very High Risk	Extremely High Risk
Total Cholesterol	< 200	< 200	200 – 239	≥ 240	≥ 240
Triglycerides	<150	< 150	150 – 199	200 – 499	≥ 500
LDL	< 100	100 – 129	130 – 159 Target for High Risk: < 70	160 – 189 Target for Very High Risk: < 50	≥ 190 Target for Extreme Risk – Category A,B :< 30,Category C: 10 – 15
HDL	≥ 60	≥ 60	M: <40, F: <50	<30	<30
Non-HDL Cholesterol	< 130	130 – 159	160-189 Target for High Risk: < 100	190 – 219 Target for Very High Risk: < 80	≥220 Target for Extreme Risk – Category A,B :< 60, Category C: 40 – 45

Note: Low risk – No known risk factor of cardiovascular disease. **Moderate risk** – Any one risk factor, eg; smoking/hypertension/diabetes mellitus etc. **High risk** – Two or more risk factors without any disease manifestation, chronic kidney disease, long-standing diabetes mellitus existing for >10 years, history of heterozygous familial hypercholesterolemia.

Very high risk – clinical evidence of coronary artery disease, long-standing diabetes mellitus existing for >20 years, etc. **Extremely high risk** – recurrent vascular events.

(Category A- CAD with >1 feature of High risk group, Category B- CAD with >1 feature of very high risk group or poly-vascular disease, Category C- Recurrent CAD with other significant risk factors as high lipoprotein a, extensive coronary calcium)

1. Measurements for Lipids (Especially Triglycerides) can show physiological (Dependent on diet,10-12 hrs fasting pre-test condition) & analytical variations.

2. Lipid Association of India (LAI) recommends screening of all adults (>20 yrs) for Atherosclerotic Cardiovascular Disease (ASCVD) risk factors with lipid profile testing.

The association recommends testing also to include Apolipoprotein B & Lipoprotein (a) for stratification and defining LDL – C targets.

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Dr. POOJITHA RAM. V
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DEPARTMENT OF BIOCHEMISTRY

APOLLO FULL BODY CHECKUP - PLATINUM WITH SMART REPORT

Test Name	Result	Unit	Bio. Ref. Interval	Method
LIVER FUNCTION TEST (LFT) , SERUM				
BILIRUBIN, TOTAL	0.34	mg/dL	0.10-1.20	Diazotized 2,4? Dichloroaniline
BILIRUBIN CONJUGATED (DIRECT)	0.11	mg/dL	0.0-0.20	Diazotized 2,4? Dichloroaniline
BILIRUBIN (INDIRECT)	0.23	mg/dL	0.0-1.1	Calculated
ALANINE AMINOTRANSFERASE (ALT/SGPT)	8	U/L	0-34	IFCC Modified method without PLP
ASPARTATE AMINOTRANSFERASE (AST/SGOT)	16.0	U/L	0-34	IFCC
AST (SGOT) / ALT (SGPT) RATIO (DE RITIS)	2.0		<1.15	Calculated
ALKALINE PHOSPHATASE	72.00	U/L	38-126	IFCC
PROTEIN, TOTAL	7.80	g/dL	6.2-8.1	Biuret
ALBUMIN	3.70	g/dL	3.2-4.6	Bromocresol Green
GLOBULIN	4.10	g/dL	2.0-3.5	Calculated
A/G RATIO	0.9		0.9-2.0	Calculated

Comment:

LFT results reflect different aspects of the health of the liver, i.e., hepatocyte integrity (AST & ALT), synthesis and secretion of bile (Bilirubin, ALP), cholestasis (ALP, GGT), protein synthesis (Albumin) Common patterns seen:

1. Hepatocellular Injury: *AST – Elevated levels can be seen. However, it is not specific to liver and can be raised in cardiac and skeletal injuries. *ALT – Elevated levels indicate hepatocellular damage. It is considered to be most specific lab test for hepatocellular injury. Values also correlate well with increasing BMI. Disproportionate increase in AST, ALT compared with ALP. AST: ALT (ratio) – In case of hepatocellular injury AST: ALT > 1 In Alcoholic Liver Disease AST: ALT usually >2. This ratio is also seen to be increased in NAFLD, Wilson's diseases, Cirrhosis, but the increase is usually not >2. Note- If both SGPT and SGOT are within reference range then AST:ALT (De Ritis ratio) does not have any clinical significance.
2. Cholestatic Pattern: *ALP – Disproportionate increase in ALP compared with AST, ALT. ALP elevation also seen in pregnancy, impacted by age and sex. *Bilirubin (Direct) and GGT elevated- helps to establish hepatic origin.
3. Synthetic function impairment: *Albumin- Liver disease reduces albumin levels, Correlation with PT (Prothrombin Time) helps.
4. Associated tests for assessment of liver fibrosis - Fibrosis-4 and APRI Index.

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APOLLO FULL BODY CHECKUP - PLATINUM WITH SMART REPORT



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DEPARTMENT OF BIOCHEMISTRY

APOLLO FULL BODY CHECKUP - PLATINUM WITH SMART REPORT

Test Name	Result	Unit	Bio. Ref. Interval	Method
RENAL PROFILE/KIDNEY FUNCTION TEST (RFT/KFT) , SERUM				
CREATININE	1.00	mg/dL	0.7-1.2	Enzymatic
eGFR - ESTIMATED GLOMERULAR FILTRATION RATE	57.68	mL/min/1.73m ²	>60	CKD-EPI Formula 2021
UREA	23.00	mg/dL	21-43	Urease with GLDH
BLOOD UREA NITROGEN	10.8	mg/dL	8.0 - 23.0	Calculated
URIC ACID	4.70	mg/dL	2.6-6	URICASE/PEROXIDASE
CALCIUM	8.80	mg/dL	8.6-10.3	Arsenazo-III
PHOSPHORUS, INORGANIC	3.60	mg/dL	2.7-4.5	PHOSPHOMOLYBDATE
SODIUM	138	mmol/L	135-145	Direct ISE
POTASSIUM	4.6	mmol/L	3.5-5.1	ISE (Indirect)
CHLORIDE	107	mmol/L	98 - 107	Direct ISE
PROTEIN, TOTAL	7.80	g/dL	6.2-8.1	Biuret
ALBUMIN	3.70	g/dL	3.2-4.6	Bromocresol Green
GLOBULIN	4.10	g/dL	2.0-3.5	Calculated
A/G RATIO	0.9		0.9-2.0	Calculated



Poojitha Ram V

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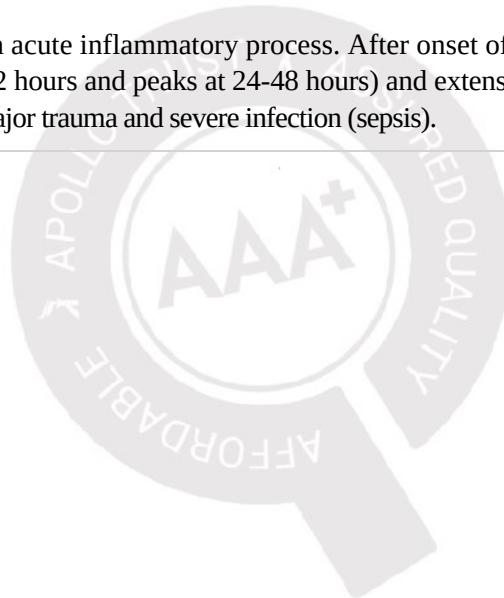
APOLLO FULL BODY CHECKUP - PLATINUM WITH SMART REPORT

Test Name	Result	Unit	Bio. Ref. Interval	Method
C-REACTIVE PROTEIN CRP (QUANTITATIVE) , SERUM	15.5	mg/L	0-5	Latex-enhanced Immunoturbidimetric

Comment:

C-reactive protein (CRP) is one of the most sensitive acute-phase reactants for inflammation. Measuring changes in the concentration of CRP provides useful diagnostic information about the level of acuity and severity of a disease. Unlike ESR, CRP levels are not influenced by hematologic conditions such as anemia, polycythemia etc.

Increased levels are consistent with an acute inflammatory process. After onset of an acute phase response, the serum CRP concentration rises rapidly (within 6-12 hours and peaks at 24-48 hours) and extensively. Concentrations above 100 mg/L are associated with severe stimuli such as major trauma and severe infection (sepsis).



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APOLLO FULL BODY CHECKUP - PLATINUM WITH SMART REPORT

Test Name	Result	Unit	Bio. Ref. Interval	Method
ESTIMATED GLOMERULAR FILTRATION RATE (EGFR) WITH CREATININE , SERUM				
CREATININE	1.00	mg/dL	0.7-1.2	Enzymatic
eGFR - ESTIMATED GLOMERULAR FILTRATION RATE	57.68	mL/min/1.73m ²	>60	CKD-EPI Formula

Comment:

This table shows prognosis of chronic kidney disease (CKD) by eGFR:

GFR Category	Description	GFR Range (mL/min/1.73 m ²)
G1 & G2	Normal	≥ 60
G3a	Mildly to moderately decreased	45-59
G3b	Moderately to severely decreased	30-44
G4	Severely decreased	15-29
G5	Kidney failure	< 15

Reference:

Kidney Disease: Improving Global Outcomes (KDIGO) Diabetes Work Group. (2022). KDIGO 2022 Clinical Practice Guideline for Diabetes Management in Chronic Kidney Disease. Kidney International, 102(5S), S1-S127.
<https://doi.org/10.1016/j.kint.2022.07.001>


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DEPARTMENT OF BIOCHEMISTRY

APOLLO FULL BODY CHECKUP - PLATINUM WITH SMART REPORT

Test Name	Result	Unit	Bio. Ref. Interval	Method
ALBUMIN (MICROALBUMIN) /CREATININE RATIO - URINE , URINE				
ALBUMIN- SPOT URINE	75.700	mg/L	<20	Immunoturbidimetric
CREATININE - URINE	132	mg/dL	28 - 217	Enzyme Kinetic
URINE ALBUMIN/CREATININE RATIO	57.35	ug/mg	<30	Calculated

Comment:

American Diabetes Association Definition of Microalbuminuria

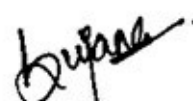
CATEGORY	MICROALBUMIN 24 HOUR URINE (mg/24 hour)	MICROALBUMIN (MAU)/CREATININE RATIO (µg/mg of creatinine)
NORMAL	< 30	< 30
MICROALBUMINURIA	30- 300	30- 300
CLINICAL ALBUMINURIA	> 300	> 300

Measurement of the urine albumin-to-creatinine ratio is recommended by the American Diabetes Association to screen for microalbuminuria.

Increased excretion of albumin (microalbuminuria) is a predictor of future development of clinical renal disease in patients with hypertension or DM.

Microalbuminuria may be seen transiently during pregnancy, after exercise, and with protein loading, hyperglycemia, fever, and urinary tract infections. There is also day-to-day, as well as diurnal, variation in albumin excretion. Hence, it is important to base treatment on the results of several tests. Vigorous exercise can cause a transient increase in albumin excretion. Patients should refrain from vigorous exercise in the 24 hours prior to the test.


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Dr.Matta Sujana Reddy
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SIN No:BI34067465

This test has been performed at Apollo Health & Life Care Pvt. Ltd., Global Reference Laboratory, Hyderabad

(CIN - U85110TG2000PLC115819)

Corporate Office: 7-1-617/A, 7th Floor, Imperial Towers, Ameerpet, Hyderabad-500016, Telangana

Ph No: 040-4904 7777 | www.apollohl.com | Email ID:enquiry@apollohl.com

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APOLLO FULL BODY CHECKUP - PLATINUM WITH SMART REPORT

Test Name	Result	Unit	Bio. Ref. Interval	Method
GLUCOSE, FASTING	170	mg/dL	70-100	GOD - POD
BETA CELL FUNCTION (%B)	32.5	%	>87.12	CALCULATED
INSULIN SENSITIVITY (%S)	69.6	%	>53.9	CALCULATED
HOMA2 IR INDEX	1.44		<1.69	CALCULATED
QUICKI	0.31		>0.45	CALCULATED

Comment:

HOMA2 IR INDEX	INTERPRETATION	QUICKI	INTERPRETATION
<1.69	Normal Insulin Resistance	>0.45	Normal, non- diabetic
1.69-2.72	Moderate Insulin Resistance	0.3 - 0.45	Insulin resistance likely
>2.72	Severe Insulin Resistance	<0.30	Diabetes diagnosis likely

HOMA2-IR (Homeostatic Model Assessment for Insulin Resistance):

- HOMA2 is the updated HOMA model where HOMA2 calculator (version 2.2) is used to calculate HOMA2 IR index, beta cell function (%B) and insulin sensitivity (%S) from fasting glucose and insulin concentrations.
- HOMA2 IR index is used to measure severity of insulin resistance, though normal insulin resistance varies depending on the population It is most useful for epidemiologic, population-based, and other group level assessments.
- HOMA2-IR index is an alternative for estimating insulin resistance (IR), has been examined in multiple studies. Results show reasonable correlation between HOMA2-IR and 'clamp' measurement, the gold standard.
- HOMA2-IR index should not be used in patients on insulin, and studies have shown its limited accuracy in those with impaired glucose tolerance, normal BMI and elderly.

QUICKI (Quantitative Insulin Sensitivity Check Index):

- QUICKI is an index used to assess insulin resistance, just like HOMA2-IR from fasting glucose and insulin concentrations.
- The smaller the QUICKI index is, the larger the probability there is a carbohydrate metabolism disorder.
- QUICKI has been shown to be a useful index of insulin sensitivity in various populations, including those with hypertension and type 2 diabetes.

Reference:

<https://www.rdm.ox.ac.uk/about/our-facilities-and-units/DTU/software/homa>
<https://www.omnicalculator.com/health/quick>

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Dr. RAJESH BATTINA
PhD.(Biochemistry)
Consultant Biochemist



Dr. Matta Sujana Reddy
M.B.B.S,M.D(Biochemistry)
Consultant Biochemist



SIN No:BI34067462

This test has been performed at Apollo Health & Lifestyle Ltd, Global Reference Laboratory,Hyderabad

Apollo Health and Lifestyle Limited

(CIN - U85110TG2000PLC115819)

Corporate Office: 7-1-617/A, 7th Floor, Imperial Towers, Ameerpet, Hyderabad-500016, Telangana

Ph No: 040-4904 7777 | www.apollohl.com | Email ID:enquiry@apollohl.com

Patient Name : Ms.VENKATAMMA G
Age/Gender : 77 Y 11 M 15 D /F
UHID/MR No : APJ1.0031336678
Visit ID : DTPTOPV38584
Ref Doctor : Dr.SELF
IP/OP NO :

Collected : 17/Aug/2026 08:23AM
Received : 17/Aug/2026 01:19PM
Reported : 17/Aug/2026 02:46PM
Status : Final Report
Client Name : APOLLO 24X7
Center location : ANNAMAYYA CIRCLE,Tirupathi

DEPARTMENT OF IMMUNOLOGY

APOLLO FULL BODY CHECKUP - PLATINUM WITH SMART REPORT

Test Name	Result	Unit	Bio. Ref. Interval	Method
TSH (Ultrasensitive/4thGen) , SERUM	3.006	µIU/mL	0.35-4.75	CLIA

Comment:

TSH is a glycoprotein hormone secreted by the anterior pituitary. TSH is a labile hormone & is secreted in a pulsatile manner throughout the day and is subject to several non-thyroidal pituitary influences. Significant variations in TSH can occur with circadian rhythm, hormonal status, stress, sleep deprivation, caloric intake, medication & circulating antibodies.

It is important to confirm any TSH abnormality in a fresh specimen drawn after ~ 3 weeks before assigning a diagnosis, as the cause of an isolated TSH abnormality.

For pregnant females	Bio Ref Range for TSH in uIU/ml (As per American Thyroid Association)
First trimester	0.1 - 2.5
Second trimester	0.2 – 3.0
Third trimester	0.3 – 3.0

Test Name	Result	Unit	Bio. Ref. Interval	Method
FREE T3 (FT3) , SERUM	3.19	pg/mL	2.5-3.9	CLIA

Comment:

Free T3 gives corrected values in patients in whom the total T3 is altered on account of changes in serum proteins or in binding sites (e.g., pregnancy), drugs (e.g., androgens, estrogens, birth control pills, phenytoin [Dilantin]), altered levels of serum proteins (e.g., nephrosis).

Elevated concentrations of T3 occur in Grave's disease and most other classical causes of hyperthyroidism.

Decreased concentrations occur in primary hypothyroid diseases such as Hashimoto thyroiditis and neonatal hypothyroidism or secondary hypothyroidism due to defects at the hypothalamohypophyseal level. It may decrease by $\leq 25\%$ in healthy older persons while FT4 remains normal.

Test Name	Result	Unit	Bio. Ref. Interval	Method
FREE T4 (FT4) , SERUM	1.01	ng/dL	0.777-1.7094	CLIA

Comment:

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Dr. POOJITHA RAM. V
M.B.B.S., M.D (Pathology)
consultant pathologist

SIN No:IM12558576



Patient Name : Ms.VENKATAMMA G
Age/Gender : 77 Y 11 M 15 D /F
UHID/MR No : APJ1.0031336678
Visit ID : DTPTOPV38584
Ref Doctor : Dr.SELF
IP/OP NO :

Collected : 17/Aug/2026 08:23AM
Received : 17/Aug/2026 01:19PM
Reported : 17/Aug/2026 02:46PM
Status : Final Report
Client Name : APOLLO 24X7
Center location : ANNAMAYYA CIRCLE,Tirupathi

DEPARTMENT OF IMMUNOLOGY

APOLLO FULL BODY CHECKUP - PLATINUM WITH SMART REPORT

FT4 gives corrected values in patients in whom the total T4 is altered on account of changes in serum proteins or in binding sites. Monitoring restoration to normal range is the only laboratory criterion to estimate appropriate replacement dose of levothyroxine because 6–8 weeks are required before TSH reflects these changes
FT4 assays are prone to inaccurate readings in pregnant women. Anticonvulsant drug therapy (particularly phenytoin) may result in decreased FT4 levels due to an increased hepatic metabolism and secondarily to displacement of hormone from binding sites.



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Dr. POOJITHA RAM. V
M.B.B.S.,M.D(Pathology)
consultant pathologist

SIN No:IM12558576

Patient Name : Ms.VENKATAMMA G
Age/Gender : 77 Y 11 M 15 D /F
UHID/MR No : APJ1.0031336678
Visit ID : DTPTOPV38584
Ref Doctor : Dr.SELF
IP/OP NO :

Collected : 17/Aug/2026 08:23AM
Received : 17/Aug/2026 01:19PM
Reported : 17/Aug/2026 02:46PM
Status : Final Report
Client Name : APOLLO 24X7
Center location : ANNAMAYYA CIRCLE,Tirupathi

DEPARTMENT OF IMMUNOLOGY

APOLLO FULL BODY CHECKUP - PLATINUM WITH SMART REPORT

Test Name	Result	Unit	Bio. Ref. Interval	Method
VITAMIN D (25 - OH VITAMIN D) , SERUM	4.58	ng/mL	30-100	CLIA

Comment:

BIOLOGICAL REFERENCE RANGES

VITAMIN D STATUS	VITAMIN D 25 HYDROXY (ng/mL)
DEFICIENCY	<10
INSUFFICIENCY	10 – 30
SUFFICIENCY	30 – 100
TOXICITY	>100

The biological function of Vitamin D is to maintain normal levels of calcium and phosphorus absorption. 25-Hydroxy vitamin D is the storage form of vitamin D. Vitamin D assists in maintaining bone health by facilitating calcium absorption. Vitamin D deficiency can also cause osteomalacia, which frequently affects elderly patients.

Vitamin D Total levels are composed of two components namely 25-Hydroxy Vitamin D2 and 25-Hydroxy Vitamin D3 both of which are converted into active forms. Vitamin D2 level corresponds with the exogenous dietary intake of Vitamin D rich foods as well as supplements. Vitamin D3 level corresponds with endogenous production as well as exogenous diet and supplements.

Vitamin D from sunshine on the skin or from dietary intake is converted predominantly by the liver into 25-hydroxy vitamin D, which has a long half-life and is stored in the adipose tissue. The metabolically active form of vitamin D, 1,25-di-hydroxy vitamin D, which has a short life, is then synthesized in the kidney as needed from circulating 25-hydroxy vitamin D. The reference interval of greater than 30 ng/mL is a target value established by the Endocrine Society.

Decreased Levels:- Inadequate exposure to sunlight, Dietary deficiency, Vitamin D malabsorption, Severe Hepatocellular disease., Drugs like Anticonvulsants, Nephrotic syndrome.

Increased levels:- Vitamin D intoxication.

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Dr. POOJITHA RAM. V
M.B.B.S.,M.D(Pathology)
consultant pathologist

SIN No:IM12558576



Patient Name : Ms.VENKATAMMA G
Age/Gender : 77 Y 11 M 15 D /F
UHID/MR No : APJ1.0031336678
Visit ID : DTPTOPV38584
Ref Doctor : Dr.SELF
IP/OP NO :

Collected : 17/Aug/2026 08:23AM
Received : 17/Aug/2026 01:19PM
Reported : 17/Aug/2026 02:47PM
Status : Final Report
Client Name : APOLLO 24X7
Center location : ANNAMAYYA CIRCLE,Tirupathi

DEPARTMENT OF IMMUNOLOGY

APOLLO FULL BODY CHECKUP - PLATINUM WITH SMART REPORT

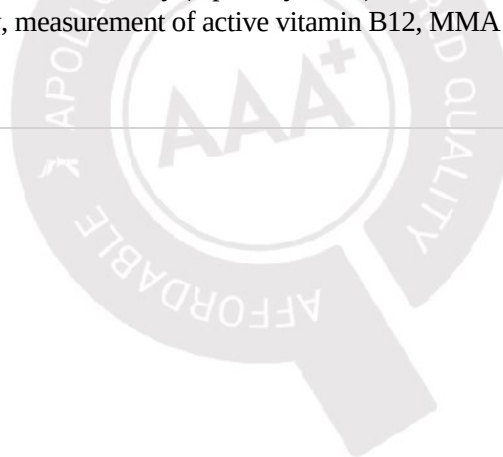
Test Name	Result	Unit	Bio. Ref. Interval	Method
VITAMIN B12 , SERUM	501	pg/mL	183-822	CLIA

Comment:

Population based data reflecting exact scenario of vitamin B12 levels in Indian population is still evolving, however, different studies reporting a deficiency in adults, pregnant women and children ranging from 16% to 77% with average of about 47%. This high incidence is attributed to vegetarian food habits of large majority of Indian population.

Vitamin B12 deficiency frequently causes macrocytic anemia, glossitis, peripheral neuropathy, weakness, hyperreflexia, ataxia, loss of proprioception, poor coordination, and affective behavioral changes. A significant increase in RBC MCV may be an important indicator of vitamin B12 deficiency. B12 levels in the range of 150 to 190 pg/ml may not be associated with any clinical manifestations, while B12 levels below 100 pg/ml are often associated with clinical symptoms. However, for an individual based on other co-morbid conditions or other nutritional deficiency (especially folate) the manifestations can vary accordingly.

If clinical symptoms suggest deficiency, measurement of active vitamin B12, MMA and homocysteine should be considered as further workup.



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Dr. POOJITHA RAM. V
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SIN No:IM12558576



Patient Name : Ms.VENKATAMMA G
Age/Gender : 77 Y 11 M 15 D /F
UHID/MR No : APJ1.0031336678
Visit ID : DTPTOPV38584
Ref Doctor : Dr.SELF
IP/OP NO :

Collected : 17/Aug/2026 08:23AM
Received : 18/Aug/2026 11:22AM
Reported : 18/Aug/2026 03:41PM
Status : Final Report
Client Name : APOLLO 24X7
Center location : ANNAMAYYA CIRCLE,Tirupathi

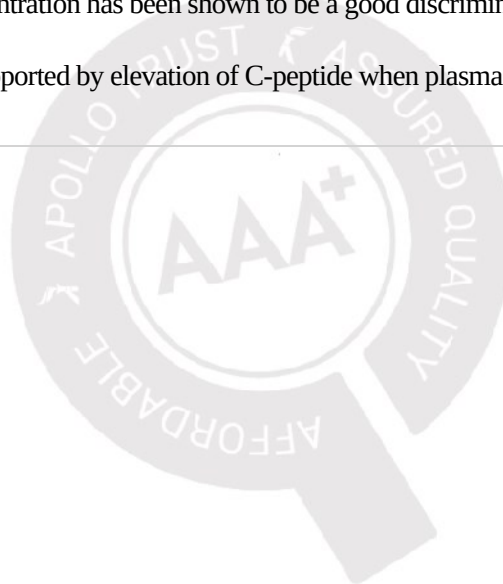
DEPARTMENT OF IMMUNOLOGY

APOLLO FULL BODY CHECKUP - PLATINUM WITH SMART REPORT

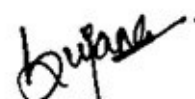
Test Name	Result	Unit	Bio. Ref. Interval	Method
C - PEPTIDE (FASTING) , SERUM	2.760	ng/mL	0.727 - 3.68	Immunometric

Comment:

1. Clinical use of C-peptide is in the evaluation of hypoglycemia (insulinoma) and in distinguishing type 1 from type 2 diabetes mellitus (DM) in ambiguous cases.
2. Patients with insulin-secreting neoplasms have high levels of both C-peptide and endogenous insulin; in contrast, patients with exogenous insulin administration as a cause of hypoglycemia will have low C-peptide levels
3. Glucagon-stimulated C-peptide concentration has been shown to be a good discriminator between insulin-requiring and non-insulin-requiring diabetic patients.
4. The diagnosis of islet cell tumor is supported by elevation of C-peptide when plasma glucose is low.




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Consultant biochemist


Dr.Matta Sujana Reddy
M.B.B.S.,M.D(Biochemistry)
Consultant Biochemist

Page 17 of 20
CAP
ACCREDITED
COLLEGE of AMERICAN PATHOLOGISTS



SIN No:IM12558577

This test has been performed at Apollo Health & Life Care Pvt. Ltd. Global Reference Laboratory, Hyderabad

(CIN - U85110TG2000PLC115819)

Corporate Office: 7-1-617/A, 7th Floor, Imperial Towers, Ameerpet, Hyderabad-500016, Telangana

Ph No: 040-4904 7777 | www.apollohl.com | Email ID:enquiry@apollohl.com

Patient Name : Ms.VENKATAMMA G
Age/Gender : 77 Y 11 M 15 D /F
UHID/MR No : APJ1.0031336678
Visit ID : DTPTOPV38584
Ref Doctor : Dr.SELF
IP/OP NO :

Collected : 17/Aug/2026 08:23AM
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Reported : 18/Aug/2026 01:40PM
Status : Final Report
Client Name : APOLLO 24X7
Center location : ANNAMAYYA CIRCLE,Tirupathi

DEPARTMENT OF IMMUNOLOGY

APOLLO FULL BODY CHECKUP - PLATINUM WITH SMART REPORT

Test Name	Result	Unit	Bio. Ref. Interval	Method
INSULIN (FASTING) , SERUM	9.63	µIU/mL	2.6-24.9	ECLIA

Comment:

This test is useful in the evaluation of fasting hypoglycemia, Diabetes Mellitus, insulin resistance and selection of optimal therapy for Diabetes. Circulating anti-insulin antibodies are often found in patients who have been treated with nonhuman forms of insulin. If present, these antibodies may interfere with the assay.

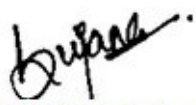
For individuals who are significantly overweight, fasting insulin levels are typically somewhat higher than for adults of normal weight. Stimulation of insulin secretion may be caused by many factors like hyperglycemia, glucagon, amino acids, growth hormone and catecholamines. A single random blood sample for insulin may provide insufficient information due to wide variation in the time responses of insulin levels and blood glucose.

Conditions associated with increased insulin include the following:

- Obesity
- Steroid administration
- Acromegaly
- Cushing syndrome
- Insulin receptor mutation
- Type 2 diabetes (early stage)

Conditions associated with decreased insulin include the following:

- Insulinoma
- Non-beta cell tumors
- Excessive insulin administration
- Insulin secretagogue administration (sulfonylurea and glinides)
- Congenital hyperinsulinism (mutation in insulin-secreting gene)
- Autoimmunity to insulin or insulin receptor



Dr. Matta Sujana Reddy
M.B.B.S, M.D (Biochemistry)
Consultant Biochemist

SIN No: IM12558577

This test has been performed at Apollo Health & Lifestyle Ltd, Global Reference Laboratory, Hyderabad



Patient Name : Ms.VENKATAMMA G
Age/Gender : 77 Y 11 M 15 D /F
UHID/MR No : APJ1.0031336678
Visit ID : DTPTOPV38584
Ref Doctor : Dr.SELF
IP/OP NO :

Collected : 17/Aug/2026 08:23AM
Received : 17/Aug/2026 02:13PM
Reported : 17/Aug/2026 02:34PM
Status : Final Report
Client Name : APOLLO 24X7
Center location : ANNAMAYYA CIRCLE,Tirupathi

DEPARTMENT OF CLINICAL PATHOLOGY

APOLLO FULL BODY CHECKUP - PLATINUM WITH SMART REPORT

Test Name	Result	Unit	Bio. Ref. Interval	Method
COMPLETE URINE EXAMINATION (CUE) , URINE				
Physical Examination				
COLOUR	PALE YELLOW		PALE YELLOW	Visual
TRANSPARENCY	HAZY		CLEAR	Physical Measurement
pH	5.00		5-7.5	Double Indicator
SP. GRAVITY	1.030		1.002-1.030	Protein Error of Indicator
BIOCHEMICAL EXAMINATION				
URINE PROTEIN	POSITIVE (+)		NEGATIVE	Protein Error of Indicator/Sulpho salicylic Acid
GLUCOSE	POSITIVE (+)		NEGATIVE	Glucose oxidase/Benedicts Test
URINE BILIRUBIN	NEGATIVE		NEGATIVE	AZO coupling reaction/Fouchets
URINE KETONES (RANDOM)	NEGATIVE		NEGATIVE	Sodium nitro prusside/Rotheras Test
UROBILINOGEN	INCREASED		NORMAL	Modified Ehrlichs
NITRITE	NEGATIVE		NEGATIVE	Diazotization
LEUCOCYTE ESTERASE	POSITIVE		NEGATIVE	Leucocyte Esterase
CENTRIFUGED SEDIMENT WET MOUNT AND MICROSCOPY				
Pus Cells	PLENTY	/hpf	0-5	Microscopy
EPITHELIAL CELLS	8-10	/hpf	<10	Microscopy
RBC	NIL	/hpf	0-2	Microscopy
CASTS	NIL		0-2 Hyaline Cast	Microscopy
CRYSTALS	ABSENT		ABSENT	Microscopy

Comment:

All urine samples are checked for adequacy and suitability before examination. All abnormal chemical examination are rechecked and verified by manual methods. Microscopy findings are reported as an average of 10 high power fields.

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Dr. POOJITHA RAM. V
M.B.B.S.,M.D(Pathology)
consultant pathologist

SIN No:C04859861



Patient Name : Ms.VENKATAMMA G
Age/Gender : 77 Y 11 M 15 D /F
UHID/MR No : APJ1.0031336678
Visit ID : DTPTOPV38584
Ref Doctor : Dr.SELF
IP/OP NO :

Collected : 17/Aug/2026 08:23AM
Received : 17/Aug/2026 02:13PM
Reported : 17/Aug/2026 02:34PM
Status : Final Report
Client Name : APOLLO 24X7
Center location : ANNAMAYYA CIRCLE,Tirupathi

DEPARTMENT OF CLINICAL PATHOLOGY

APOLLO FULL BODY CHECKUP - PLATINUM WITH SMART REPORT

***** End Of Report *****



Page 20 of 20



Dr. POOJITHA RAM. V
M.B.B.S., M.D (Pathology)
consultant pathologist

SIN No: C04859861



Patient Name	: Ms.VENKATAMMA G	Collected	: 17/Aug/2026 08:23AM
Age/Gender	: 77 Y 11 M 15 D /F	Received	: 17/Aug/2026 02:13PM
UHID/MR No	: APJ1.0031336678	Reported	: 17/Aug/2026 02:34PM
Visit ID	: DTPTOPV38584	Status	: Final Report
Ref Doctor	: Dr.SELF	Client Name	: APOLLO 24X7
IP/OP NO	:	Center location	: ANNAMAYYA CIRCLE,Tirupathi

TERMS AND CONDITIONS GOVERNING THIS REPORT

1. Reported results are for information and interpretation of the referring doctor or such other medical professionals, who understand reporting units, reference ranges and limitation of technologies. Laboratories not be responsible for any interpretation whatsoever
2. This is computer generated medical diagnostics report that has been validated by an Authorized Medical Practitioner/Doctor. The report does not need physical signature.
3. Partial reproduction of this report is not valid and should not be resorted to draw any conclusion.
4. In the case you are not the intended recipient of this report. Please immediately return the same to the concerned issuing desk. Any disclosure, copy or distribution of any contents of this report, is unlawful and is strictly prohibited.
5. Results delays may occur due to unforeseen circumstances such as non-availability of kits, equipment breakdown, natural calamities, IT downtime, logistic delays or any other unavoidable event. For certain tests based on analyte stability, criticality of results and in the interest of patient for having appropriate medical diagnosis, the same test may be outsourced to other accredited laboratory.
6. It is presumed that the tests performed are, on the specimen / sample being to the patient named or identified and the verifications of particulars have been confirmed by the patient or his / her representative at the point of generation of said specimen
7. The reported results are restricted to the given specimen only. Results may vary from lab to lab and from time to time for the same parameter for the same patient (within subject biological variation).
8. The patient details along with their results in certain cases like notifiable diseases and as per local regulatory requirements will be communicated to the assigned regulatory bodies
9. The patient samples can be used as part of internal quality control, test verification, data analysis purposes within the testing scope of the laboratory.
10. This report is not valid for medico legal purposes. It is performed to facilitate medical diagnosis only


Dr. POOJITHA RAM. V
M.B.B.S,M.D(Pathology)
consultant pathologist



SIN No:C04859861

Apollo Health and Lifestyle Limited

(CIN - U85110TG2000PLC115819)

Corporate Office: 7-1-617/A, 7th Floor, Imperial Towers, Ameerpet, Hyderabad-500016, Telangana

Ph No: 040-4904 7777 | www.apollohl.com | Email ID:enquiry@apollohl.com

Smart Health Report

Helping you understand
your health better

Curated for
Venkatamma G

Uhid
APJ1.0031336678

Curated Date
18-08-2026



Contents	Page no.
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Body systems in detail	
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Parameters in detail	<u>36</u> ↗
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Dear Venkatamma G,

You've just completed your HbA1c (Glycated Hemoglobin), Liver Function Test (Lft), 25-hydroxy Vitamin D (D2+D3), Vitamin B12, Lipid Profile etc tests, and we've updated your health summary with the latest insights. Regular check-ins like this help you stay informed and confident about your health—great job taking this step.

OVERALL HEALTH SUMMARY

Focus on blood sugar, kidney, and blood health.

Your recent health review provides a clear picture of your current well-being, highlighting areas where focused attention can support your overall health.

Key insights from your recent tests include:

- Your **HbA1c** and **Estimated Average Glucose** levels are elevated, indicating that your blood sugar management needs closer attention. This suggests that your diabetes is currently not well-controlled, and adjusting your management plan could be beneficial.
- Your **hemoglobin**, **MCH**, **MCV**, and **PCV** levels are low, along with an elevated **RDW-CV**, which points to microcytic anemia. This means your red blood cells are smaller than usual and you have fewer of them, which can lead to fatigue and weakness.
- Your **eGFR** indicates a mild to moderate reduction in **kidney function**. This is an important area to monitor, especially given your hypertension and elevated blood sugar, as these conditions can impact kidney health.
- You have a significant deficiency in **Vitamin D**. Adequate Vitamin D is crucial for bone health, immune function, and overall well-being.
- Your **C-Reactive Protein** is elevated, suggesting some level of inflammation in your body.

YOUR CLINICAL PROFILE

The medical history powering your personalised health insights

HEALTH CONDITIONS

Active Conditions

**Hypertension**

Inferred from your medicines

Missing a recent diagnosis or past health condition? [Update Conditions →](#)

MEDICINE DETAILS

Past Medications

Ln Beta 5 Tablet

28 Mar'25 - 28 Mar'26

Started a new medicine or
changed your current dosage?

[Update Medicines →](#)

SURGICAL HISTORY

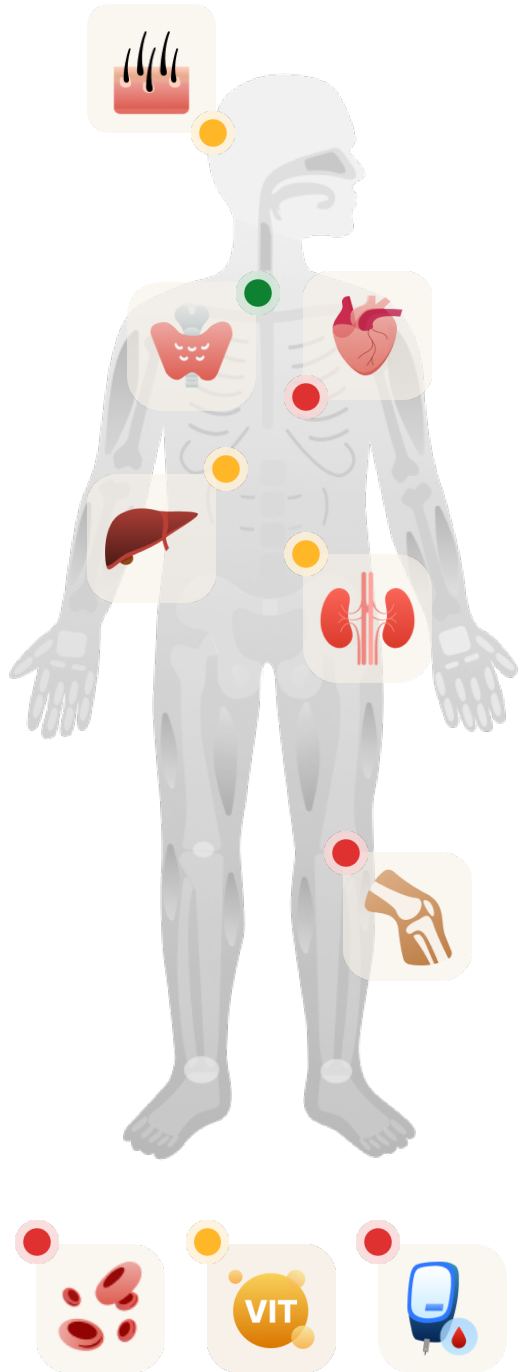
Have you had any past surgeries or medical procedures? [Add Surgery →](#)

ALLERGIES

Do you have any known food, drug, or environmental allergies?

[Add Allergy →](#)

Your body systems — Simplified



Heart Health*



NEEDS ATTENTION
As on 17th Aug, '26



Blood Health*



NEEDS ATTENTION
As on 17th Aug, '26



Blood Glucose*



NEEDS ATTENTION
As on 17th Aug, '26



Bone & Muscle



NEEDS ATTENTION
As on 17th Aug, '26



Kidney Health*



NEEDS MONITORING
As on 17th Aug, '26



Skin & Hair



NEEDS MONITORING
As on 17th Aug, '26



Vitamins*



NEEDS MONITORING
As on 17th Aug, '26



GI & Liver Health*



NEEDS MONITORING
As on 17th Aug, '26

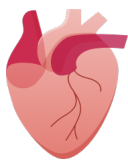


Hormone Health*



IDEAL HEALTH
As on 17th Aug, '26

* THIS INSIGHT IS BASED ON INCOMPLETE TEST DATA. SOME KEY PARAMETERS NEEDED TO FULLY ASSESS THIS ORGAN'S HEALTH WERE NOT AVAILABLE.



Heart Health

NEEDS ATTENTION As on 17th Aug, '26

Focus on improving your cholesterol levels

Your heart health needs attention due to several elevated cholesterol markers, which can put strain on your heart, especially with your history of Hypertension.

- Non-HDL Cholesterol 136 mg/dL, Outside the normal range; may need medical attention.
- Triglycerides - Serum 297 mg/dL, Outside the normal range; may need medical attention.
- Total Cholesterol / HDL Ratio 5.53, Outside the normal range; may need medical attention.

NOTE : This information is for educational purposes only. Please consult your doctor for personalised advice.

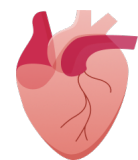
Here are a few Next Steps for you:

- 1 Schedule your Lipid Profile by 2027-02-13 to recheck your Non-HDL Cholesterol levels. [View recommended tests](#) ↗
- 2 Engage in regular physical activity, such as brisk walking for 30 minutes most days, to help improve your cholesterol levels and support overall heart health.
- 3 Focus on a diet rich in fruits, vegetables, and whole grains, while limiting saturated and trans fats, to help manage your cholesterol.

79%

Heart Health assessment is based on partial data

Some key tests are missing or old. Getting these done can provide a more accurate health picture



Heart Health

NEEDS ATTENTION As on 17th Aug, '26

DIRECT TESTS:

Here are the tests that affect this organ function directly. Monitoring these are critical to understanding Heart Health

Lipids			• 4 OUT OF RANGE ✓ 3 IN RANGE
PARAMETER NAME	RECENT VALUE	PAST VALUE	
Non-HDL Cholesterol	● 136 mg/dL on 17th Aug, '26	N/A	
Triglycerides - Serum	● 297 mg/dL on 17th Aug, '26	N/A	
Total Cholesterol / HDL Ratio	● 5.53 on 17th Aug, '26	N/A	
VLDL Cholesterol - Serum	● 59.4 mg/dL on 17th Aug, '26	N/A	
Total Cholesterol - Serum	✓ 166 mg/dL on 17th Aug, '26	N/A	
HDL Cholesterol - Serum	✓ 30 mg/dL on 17th Aug, '26	N/A	
LDL Cholesterol - Serum	✓ 76.6 mg/dL on 17th Aug, '26	N/A	

INDIRECT TESTS FOR AFFECTING YOUR HEART HEALTH

Here are a few tests that might be risk markers for your Heart Health

1 Hemoglobin

Too high thickens blood, raising blood pressure and clot risk. Too low reduces oxygen supply, weakening the heart.

As on 17th Aug, 2026, this value was 10.8 g/dL out of range

2 Estimated Average Glucose (EAG)

High EAG damages blood vessels, increasing heart disease and stroke risk.

As on 17th Aug, 2026, this value was 223 mg/dL out of range

3 Hba1C or Glycated Hemoglobin

High HbA1c indicates prolonged high blood sugar, leading to artery damage and higher heart attack risk.

As on 17th Aug, 2026, this value was 9.4 % out of range

 79%

Your Heart assessment is based on partial data

Here are the key parameters needed to complete your Heart health profile:

1

These essential parameters directly measure your Heart's health. We recommend testing them to get a clear and complete picture of its current function

- Apolipoprotein B

2

These critical parameters help us evaluate key risks to your Heart from other body systems. We recommend testing them for a complete risk profile.

- Glucose, Fasting



Ready to complete your health picture?

Add all recommended tests to your cart to get an up-to-date health analysis

[Book tests now](#) ↗



Have recent reports from other labs?

Upload your existing reports to complete your health picture without needing new tests

[Upload now](#) ↗



Blood Health

NEEDS ATTENTION As on 17th Aug, '26

Let's work on your hemoglobin levels

Your blood health needs attention, primarily due to low hemoglobin, which can affect your body's oxygen transport.

- Hemoglobin 10.8 g/dL, Outside the normal range; may need medical attention.
- RDW-CV 15.8 %, Continues to stay within Borderline range. Small lifestyle changes might help improve this.
- MCH (Mean Corpuscular HB) 26 pg, Continues to stay within Borderline range. Small lifestyle changes might help improve this.

NOTE : This information is for educational purposes only. Please consult your doctor for personalised advice.

Here are a few Next Steps for you:

- 1 Schedule your Complete Blood Count (CBC) / Hemogram now to check your Hemoglobin levels. [View recommended tests](#) ↗
- 2 Ensure you get adequate rest and manage stress, as these can impact your overall blood health and energy levels.
- 3 Include iron-rich foods like lean meats, beans, and leafy greens in your diet to support healthy hemoglobin levels.

 100% Blood Health profile is complete



Blood Health

NEEDS ATTENTION As on 17th Aug, '26

DIRECT TESTS:

Here are the tests that affect this organ function directly. Monitoring these are critical to understanding Blood Health

Blood Counts			● 1 OUT OF RANGE ● 5 BORDERLINE ✓ 13 IN RANGE
PARAMETER NAME	RECENT VALUE	PAST VALUE	
Hemoglobin	● 10.8 g/dL on 17th Aug, '26	N/A	
RDW-CV	● 15.8 % on 17th Aug, '26	N/A	
MCH (Mean Corpuscular HB)	● 26 pg on 17th Aug, '26	N/A	
Packed Cell Volume (PCV)	● 33.80 % on 17th Aug, '26	N/A	
Total Leucocytes Count (WBC)	● 7,980 cells/cu.mm on 17th Aug, '26	N/A	
Mean Cell Volume (MCV)	● 81.6 fL on 17th Aug, '26	N/A	
Absolute Basophil Count (ABC)	✓ 55.86 Cells/cu.mm on 17th Aug, '26	N/A	
Absolute Monocyte Count (AMC)	✓ 470.82 Cells/cu.mm on 17th Aug, '26	N/A	
Basophils	✓ 0.7 % on 17th Aug, '26	N/A	
Absolute Lymphocyte Count (ALC)	✓ 1939.14 Cells/cu.mm on 17th Aug, '26	N/A	
Absolute Eosinophil Count (AEC)	✓ 239.4 Cells/cu.mm on 17th Aug, '26	N/A	
Lymphocytes	✓ 24.3 % on 17th Aug, '26	N/A	
Eosinophils	✓ 3 % on 17th Aug, '26	N/A	
MCHC (Mean Corpuscular HB Concentration)	✓ 31.9 g/dL on 17th Aug, '26	N/A	
Platelet Count	✓ 378000 cells/cu.mm on 17th Aug, '26	N/A	

Red Blood Cell Count	✓ 4.15 Million/cu.mm on 17th Aug, '26	N/A
Absolute Neutrophil Count (ANC)	✓ 5274.78 Cells/cu.mm on 17th Aug, '26	N/A
Monocytes	✓ 5.9 % on 17th Aug, '26	N/A
Neutrophils	✓ 66.1 % on 17th Aug, '26	N/A



Blood Glucose

NEEDS ATTENTION As on 17th Aug, '26

Let's improve your blood sugar control

Your blood glucose levels need attention, with several markers indicating suboptimal sugar control.

- Glucose, Post Prandial (Pp) 252 mg/dL, Outside the normal range; may need medical attention.
- Estimated Average Glucose (EAG) 223 mg/dL, Outside the normal range; may need medical attention.

NOTE : This information is for educational purposes only. Please consult your doctor for personalised advice.

Here are a few Next Steps for you:

- 1 Schedule your Diabetes Profile / Blood Sugar Test now to check your Glucose, Post Prandial (Pp) levels. [View recommended tests](#) ↗
- 2 Regular physical activity, even moderate exercise like walking, can significantly help in managing your blood sugar levels.
- 3 Prioritize a balanced diet with controlled portions of carbohydrates and plenty of fiber-rich foods to help stabilize your blood glucose.

 75%

Blood Glucose assessment is based on partial data

Some key tests are missing or old. Getting these done can provide a more accurate health picture



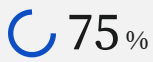
Blood Glucose

NEEDS ATTENTION As on 17th Aug, '26

DIRECT TESTS:

Here are the tests that affect this organ function directly. Monitoring these are critical to understanding Blood Glucose

Sugar		• 3 OUT OF RANGE
PARAMETER NAME	RECENT VALUE	PAST VALUE
Estimated Average Glucose (EAG)	● 223 mg/dL on 17th Aug, '26	N/A
Glucose, Post Prandial (Pp)	● 252 mg/dL on 17th Aug, '26	N/A
Hba1C or Glycated Hemoglobin	● 9.4 % on 17th Aug, '26	N/A

 75 %

Your Blood Glucose assessment is based on partial data

Here are the key parameters needed to complete your Blood Glucose health profile:

1

These essential parameters directly measure your Blood Glucose's health. We recommend testing them to get a clear and complete picture of its current function

- Glucose, Fasting



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Bone & Muscle

NEEDS ATTENTION As on 17th Aug, '26

Your Vitamin D levels need a boost

Your bone and muscle health needs attention due to very low Vitamin D levels, which are essential for strong bones.

- Vitamin D-25 Hydroxy 4.58 ng/mL, Outside the normal range; may need medical attention.
- Calcium, Serum 8.80 mg/dL, Within the healthy range.

NOTE : This information is for educational purposes only. Please consult your doctor for personalised advice.

Here are a few Next Steps for you:

- 1 Schedule your General Health Check by 2027-02-13 to recheck your Vitamin D-25 Hydroxy levels. [View recommended tests](#) ↗
- 2 Spend some time outdoors in natural sunlight, as this helps your body produce Vitamin D, which is crucial for bone health.
- 3 Incorporate foods fortified with Vitamin D, such as certain dairy products and cereals, into your diet.

 80%

Bone & Muscle assessment is based on partial data

Some key tests are missing or old. Getting these done can provide a more accurate health picture



Bone & Muscle

NEEDS ATTENTION As on 17th Aug, '26

DIRECT TESTS:

Here are the tests that affect this organ function directly. Monitoring these are critical to understanding Bone & Muscle

Minerals			✓ 1 IN RANGE
PARAMETER NAME	RECENT VALUE	PAST VALUE	
Calcium, Serum	✓ 8.80 mg/dL on 17th Aug, '26	N/A	

Vitamins			● 1 OUT OF RANGE
PARAMETER NAME	RECENT VALUE	PAST VALUE	
Vitamin D-25 Hydroxy	● 4.58 ng/mL on 17th Aug, '26	N/A	

 80 %

Your Bone and Muscle assessment is based on partial data

Here are the key parameters needed to complete your Bone and Muscle health profile:

1

These critical parameters help us evaluate key risks to your Bone and Muscle from other body systems. We recommend testing them for a complete risk profile.

- Parathyroid Hormone [PTH]



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Kidney Health

NEEDS MONITORING As on 17th Aug, '26

Let's monitor your kidney function

Your kidney health is borderline, with your eGFR indicating reduced filtration efficiency that may need follow-up.

- Estimated glomerular filtration rate(eGFR) 57.68 mL/min/1.73m², Outside the normal range; may need medical attention.
- Creatinine 1.00 mg/dL, Within the healthy range.
- Blood Urea Nitrogen 10.8 mg/dL, Within the healthy range.

NOTE : This information is for educational purposes only. Please consult your doctor for personalised advice.

Here are a few Next Steps for you:

- 1 Schedule your Kidney Function Test (KFT) now to check your Estimated glomerular filtration rate(eGFR) levels. [View recommended tests ↗](#)
- 2 Stay well-hydrated by drinking plenty of water throughout the day to support healthy kidney function.
- 3 Limit processed foods and reduce sodium intake to help maintain kidney health.

C 50%

Kidney Health assessment is based on partial data

Some key tests are missing or old. Getting these done can provide a more accurate health picture



Kidney Health

NEEDS MONITORING As on 17th Aug, '26

DIRECT TESTS:

Here are the tests that affect this organ function directly. Monitoring these are critical to understanding Kidney Health

Kidney Function Test (KFT)

1 OUT OF RANGE 3 IN RANGE

PARAMETER NAME	RECENT VALUE	PAST VALUE
Estimated glomerular filtration rate(eGFR)	57.68 mL/min/1.73m² on 17th Aug, '26	N/A
Creatinine	1.00 mg/dL on 17th Aug, '26	N/A
Blood Urea Nitrogen	10.8 mg/dL on 17th Aug, '26	N/A
Urea	23.00 mg/dL on 17th Aug, '26	N/A

Electrolytes

3 IN RANGE

PARAMETER NAME	RECENT VALUE	PAST VALUE
Sodium - Serum	138 mmol/L on 17th Aug, '26	N/A
Potassium - Serum	4.6 mmol/L on 17th Aug, '26	N/A
Chloride - Serum	107 mmol/L on 17th Aug, '26	N/A

Minerals

2 IN RANGE

PARAMETER NAME	RECENT VALUE	PAST VALUE
Calcium, Serum	8.80 mg/dL on 17th Aug, '26	N/A
Phosphorus, Inorganic - Serum	3.60 mg/dL on 17th Aug, '26	N/A

INDIRECT TESTS FOR AFFECTING YOUR KIDNEY HEALTH

Here are a few tests that might be risk markers for your Kidney Health

1 Estimated Average Glucose (EAG)

High EAG suggests prolonged high blood sugar, which can damage kidney blood vessels and lead to kidney disease.

*As on 17th Aug, 2026, **this value was 223 mg/dL out of range***

2 Hba1C or Glycated Hemoglobin

High HbA1c indicates chronic high blood sugar, which damages kidney filtration ability.

*As on 17th Aug, 2026, **this value was 9.4 % out of range***

 50%

Your Kidney assessment is based on partial data

Here are the key parameters needed to complete your Kidney health profile:

1

These essential parameters directly measure your Kidney's health. We recommend testing them to get a clear and complete picture of its current function

- Leukocyte Esterase
- Urine Blood
- Casts
- Epithelial Cells
- Urine Protein
- Ketones
- Crystals
- Bacteria
- Parasites

2

These critical parameters help us evaluate key risks to your Kidney from other body systems. We recommend testing them for a complete risk profile.

- Red Blood Cells
- Glucose (Urine Sugar)



Ready to complete your health picture?

Add all recommended tests to your cart to get an up-to-date health analysis

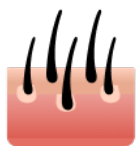
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Skin & Hair

NEEDS MONITORING As on 17th Aug, '26

Let's monitor your WBC count

Your skin and hair health is borderline, with your Total Leucocytes Count (WBC) showing mild changes that may need follow-up.

- Total Leucocytes Count (WBC) 7,980 cells/cu.mm, Continues to stay within Borderline range. Small lifestyle changes might help improve this.

NOTE : This information is for educational purposes only. Please consult your doctor for personalised advice.

Here are a few Next Steps for you:

- 1 Schedule your Complete Blood Count (CBC) / Hemogram now to check your Total Leucocytes Count (WBC) levels.

[View recommended tests](#) ↗

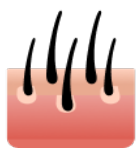
- 2 Ensure you get adequate sleep and manage stress, as these factors can influence your overall immune system and skin health.

- 3 Maintain a balanced diet rich in vitamins and minerals to support healthy skin and hair.

 75%

Skin & Hair assessment is based on partial data

Some key tests are missing or old. Getting these done can provide a more accurate health picture



Skin & Hair

NEEDS MONITORING As on 17th Aug, '26

DIRECT TESTS:

Here are the tests that affect this organ function directly. Monitoring these are critical to understanding Skin & Hair

Blood Counts			1 BORDERLINE
PARAMETER NAME	RECENT VALUE	PAST VALUE	
Total Leucocytes Count (WBC)	7,980 cells/cu.mm on 17th Aug, '26	N/A	

INDIRECT TESTS FOR AFFECTING YOUR SKIN & HAIR

Here are a few tests that might be risk markers for your Skin & Hair

1 Hemoglobin

Low hemoglobin (anemia) reduces oxygen supply to hair follicles and skin, causing hair thinning, dull skin, and increased hair loss.

As on 17th Aug, 2026, **this value was 10.8 g/dL out of range**

2 Vitamin D-25 Hydroxy

As on 17th Aug, 2026, **this value was 4.58 ng/mL out of range**

 75%

Your Skin and Hair assessment is based on partial data

Here are the key parameters needed to complete your Skin and Hair health profile:

1

These essential parameters directly measure your Skin and Hair's health. We recommend testing them to get a clear and complete picture of its current function

- Testosterone serum



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Vitamins

NEEDS MONITORING As on 17th Aug, '26

Your Vitamin D levels need a boost

Your vitamin levels are borderline, primarily due to very low Vitamin D, which is crucial for many bodily functions.

- Vitamin D-25 Hydroxy 4.58 ng/mL, Outside the normal range; may need medical attention.
- Vitamin B12 -Serum 501 pg/mL, Within the healthy range.
- Calcium, Serum 8.80 mg/dL, Within the healthy range.

NOTE : This information is for educational purposes only. Please consult your doctor for personalised advice.

Here are a few Next Steps for you:

- 1 Schedule your General Health Check by 2027-02-13 to recheck your Vitamin D-25 Hydroxy levels. [View recommended tests ↗](#)
- 2 Regular sun exposure, in moderation, can help your body produce Vitamin D naturally.
- 3 Incorporate Vitamin D-rich foods like fatty fish, fortified milk, and cereals into your diet.

C 60%

Vitamins assessment is based on partial data

Some key tests are missing or old. Getting these done can provide a more accurate health picture



Vitamins

NEEDS MONITORING As on 17th Aug, '26

DIRECT TESTS:

Here are the tests that affect this organ function directly. Monitoring these are critical to understanding Vitamins

Minerals			✓ 1 IN RANGE
PARAMETER NAME	RECENT VALUE	PAST VALUE	
Calcium, Serum	✓ 8.80 mg/dL on 17th Aug, '26	N/A	
Vitamins			● 1 OUT OF RANGE ✓ 1 IN RANGE
PARAMETER NAME	RECENT VALUE	PAST VALUE	
Vitamin D-25 Hydroxy	● 4.58 ng/mL on 17th Aug, '26	N/A	
Vitamin B12 -Serum	✓ 501 pg/mL on 17th Aug, '26	N/A	

C60%

Your Vitamins assessment is based on partial data

Here are the key parameters needed to complete your Vitamins health profile:

- 1
- These essential parameters directly measure your Vitamins's health. We recommend testing them to get a clear and complete picture of its current function

• Serum Zinc

• Serum Iron



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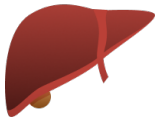
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GI & Liver Health

NEEDS MONITORING As on 17th Aug, '26

Let's monitor your Globulin levels

Your GI and liver health is borderline, with your Globulin levels slightly outside the normal range.

- Globulin 4.10 g/dL, Outside the normal range; may need medical attention.
- Bilirubin Direct 0.11 mg/dL, Within the healthy range.
- Bilirubin-Indirect 0.23 mg/dL, Within the healthy range.

NOTE : This information is for educational purposes only. Please consult your doctor for personalised advice.

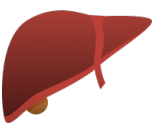
Here are a few Next Steps for you:

- 1 Schedule your Liver Function Test (LFT) now to check your Globulin levels. [View recommended tests](#) ↗
- 2 Maintain a healthy weight through regular exercise, as this can support overall liver health.
- 3 Eat a balanced diet and limit alcohol consumption to help keep your liver functioning well.

C 61%

GI & Liver Health assessment is based on partial data

Some key tests are missing or old. Getting these done can provide a more accurate health picture



GI & Liver Health

NEEDS MONITORING As on 17th Aug, '26

DIRECT TESTS:

Here are the tests that affect this organ function directly. Monitoring these are critical to understanding GI & Liver Health

Liver Function Test (LFT)			● 1 OUT OF RANGE ✓ 9 IN RANGE
PARAMETER NAME	RECENT VALUE	PAST VALUE	
Globulin	● 4.10 g/dL on 17th Aug, '26	N/A	
Bilirubin Direct	✓ 0.11 mg/dL on 17th Aug, '26	N/A	
Bilirubin-Indirect	✓ 0.23 mg/dL on 17th Aug, '26	N/A	
Bilirubin Total	✓ 0.34 mg/dL on 17th Aug, '26	N/A	
Aspartate Aminotransferase (AST / SGOT)	✓ 16.0 U/L on 17th Aug, '26	N/A	
Alkaline Phosphatase (ALP)	✓ 72.00 U/L on 17th Aug, '26	N/A	
Alanine Transaminase (SGPT / ALT)	✓ 8 U/L on 17th Aug, '26	N/A	
Albumin/Globulin Ratio	✓ 0.9 on 17th Aug, '26	0.9 on 17th Aug, '26	
Albumin	✓ 3.70 g/dL on 17th Aug, '26	N/A	
Proteins	✓ 7.80 g/dL on 17th Aug, '26	N/A	

INDIRECT TESTS FOR AFFECTING YOUR GI & LIVER HEALTH

Here are a few tests that might be risk markers for your GI & Liver Health

1 Hemoglobin

Low levels reduce oxygen to the gut, causing weak digestion and infection risk. High levels may affect blood flow, leading to digestive issues.

*As on 17th Aug, 2026, **this value was 10.8 g/dL out of range***

C 61 %

Your GI and Liver assessment is based on partial data

Here are the key parameters needed to complete your GI and Liver health profile:

1

These essential parameters directly measure your GI and Liver's health. We recommend testing them to get a clear and complete picture of its current function

- Urobilinogen
- Bile Pigment
- Urine Bilirubin
- Gamma Glutamyl Transferase (GGT)
- Amylase
- Lipase



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Hormone Health

IDEAL HEALTH As on 17th Aug, '26

Your hormone levels look great

Your hormone health is in an ideal range, with all key thyroid markers showing healthy levels.

- Free T4 (FT4) 1.01 ng/dL, Within the healthy range.
- Thyroid Stimulating Hormone (TSH), Serum 3.006 μ IU/mL, Within the healthy range.
- Free T3 (FT3) 3.19 pg/mL, Within the healthy range.

NOTE : This information is for educational purposes only. Please consult your doctor for personalised advice.

Here are a few Next Steps for you:

- 1 Continue your current medications as prescribed to maintain your excellent hormone balance.
- 2 Maintain a balanced lifestyle with regular exercise and stress management to support your overall hormonal well-being.
- 3 Continue to eat a nutritious and varied diet to support your body's hormone production and regulation.

37%

Hormone Health assessment is based on partial data

Some key tests are missing or old. Getting these done can provide a more accurate health picture



Hormone Health

IDEAL HEALTH As on 17th Aug, '26

DIRECT TESTS:

Here are the tests that affect this organ function directly. Monitoring these are critical to understanding Hormone Health

Thyroid			✓ 3 IN RANGE
PARAMETER NAME	RECENT VALUE	PAST VALUE	
Free T4 (FT4)	✓ 1.01 ng/dL on 17th Aug, '26	N/A	
Thyroid Stimulating Hormone (TSH), Serum	✓ 3.006 µIU/mL on 17th Aug, '26	N/A	
Free T3 (FT3)	✓ 3.19 pg/mL on 17th Aug, '26	N/A	

 37%

Your Hormone assessment is based on partial data

Here are the key parameters needed to complete your Hormone health profile:

1

These essential parameters directly measure your Hormone's health. We recommend testing them to get a clear and complete picture of its current function

- Testosterone serum
- Follicular Stimulating Hormone (FSH)
- Anti Thyroglobulin Antibody
- Prolactin
- Leutinising Hormone (LH)
- Parathyroid Hormone [PTH]



Ready to complete your health picture?

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Let's understand the
critical markers in
detail that require
your focus



Non-HDL Cholesterol

17th Aug, 2026

136 mg/dL

OUT OF RANGE

- Non-HDL cholesterol includes all harmful cholesterol types (LDL, VLDL, etc.). This test helps assess heart disease risk better than LDL alone. Lower levels are better for heart health.
- Low non-HDL cholesterol means fewer "harmful" particles are circulating than expected. This is usually reassuring, especially if HDL is normal. Review helps confirm overall balance.

Reference range

Your result (136 mg/dL) is **out of range high**

Inference	Value (mg/dL)
Ideal (Normal Range)	0-130



eGFR

17th Aug, 2026

57.68

mL/min/1.73m²

OUT OF RANGE

- eGFR measures how well your kidneys filter waste from the blood. It helps detect kidney disease early, especially in people with diabetes or hypertension.
- High eGFR is less common and may reflect over-estimation or early changes in kidney function. If creatinine or urea are also out of range, it adds context. Review helps confirm.

Reference range

Your result (57.68 mL/min/1.73m²) is **out of range low**

Inference	Value (ml/min/1.73 m2)
Out of range Low	<60
Borderline Low	60-90
Ideal (Normal Range)	90-120



WBC

17th Aug, 2026

7,980
cells/cu.mm
BORDERLINE

- This test counts white blood cells, which defend your body against infections and illnesses.
- High WBCs can indicate infection, inflammation, or stress; low levels suggest weakened immunity.

 **eAG**
17th Aug, 2026

223 mg/dL
OUT OF RANGE

- EAG translates HbA1c results into an estimated daily blood sugar average over 2-3 months. It helps track long-term glucose control in diabetes management.
- High EAG suggests prolonged high blood sugar, which can damage kidney blood vessels and lead to kidney disease.

Reference range

Your result (223 mg/dL) is **out of range high**

Inference	Value (mg/dL)
Out of range High	>154
Borderline High	126-154
Ideal (Normal Range)	70-126



Glucose PP

17th Aug, 2026

252 mg/dL

OUT OF RANGE

- This test measures blood sugar levels 2 hours after eating. It helps assess how the body processes sugar and diagnose diabetes or insulin resistance.
- Low PP glucose means sugar levels dropped below expected after a meal. This may cause weakness or sweating. If fasting glucose is also low, it adds clues. Review helps guide care.

Reference range

Your result (252 mg/dL) is **out of range high**

Inference	Value (mg/dL)
Out of range High	>199
Borderline High	140-199
Ideal (Normal Range)	70-140



VLDL

17th Aug, 2026

59.4 mg/dL

OUT OF RANGE

- This test measures very low-density lipoprotein (VLDL) cholesterol, a type of "bad" cholesterol. High levels are associated with an increased risk of cardiovascular disease.
- Low VLDL means fewer particles carrying triglycerides in the blood. This may follow weight loss or diet changes. If total cholesterol or HDL are also low, this adds context. Usually not urgent, but a doctor can guide if further checks are needed.

Reference range

Your result (59.4 mg/dL) is **out of range high**

Inference	Value (mg/dL)
Out of range High	>33
Borderline High	30-33
Ideal (Normal Range)	0-30



Hba1C

17th Aug, 2026

9.4 %

OUT OF RANGE

- HbA1C measures the percentage of sugar attached to hemoglobin in red blood cells. It reflects average blood sugar levels over 2-3 months and helps diagnose and monitor diabetes.
- High HbA1c indicates chronic high blood sugar, which damages kidney filtration ability.

Reference range

Your result (9.4 %) is out of range high

Inference	Value (%)
Out of range High	>6.4
Borderline High	5.7-6.4
Ideal (Normal Range)	4-5.7



MCH

17th Aug, 2026

26 pg

BORDERLINE

- This test checks how much hemoglobin your red blood cells contain on average.
- A slightly low MCH means each red cell carries a bit less hemoglobin. This may follow low iron foods or recent illness. If MCV or ferritin are also low, that supports it. Usually not urgent, but worth repeating.

Reference range

Your result (26 pg) is **borderline low**

Inference	Value (pg)
Out of range Low	<26
Borderline Low	26-27
Ideal (Normal Range)	27-31



Vitamin D-25

17th Aug, 2026

4.58 ng/mL

OUT OF RANGE

- The Vitamin D-25 Hydroxy Test assesses vitamin D status, which is crucial for calcium metabolism, immune function, and musculoskeletal health. It helps diagnose deficiency, which may lead to weak bones, or toxicity, causing calcium imbalances.
- High vitamin D usually reflects extra supplement use. If calcium is also high, this may stress the kidneys. Not usually urgent, but review helps adjust intake and prevent imbalance.

Reference range

Your result (4.58 ng/mL) is **out of range low**

Inference	Value (ng/mL)
Out of range Low	<20
Borderline Low	20-30
Ideal (Normal Range)	30-100



Globulin

17th Aug, 2026

4.10 g/dL

OUT OF RANGE

- Globulins are proteins in the blood that support immunity, transport nutrients, and help with clotting. This test helps assess immune function, liver health, and protein balance.
- Very high globulin means extra blood proteins are present. You may feel weak, swollen, or have frequent infections. If total protein or A/G ratio are also abnormal, this should be reviewed.

Reference range

Your result (4.10 g/dL) is **critical high**

Inference	Value (g/dL)
Critical High	>=3.9
Out of range High	3.6-3.9
Borderline High	3.5-3.6
Ideal (Normal Range)	2-3.5



Hemoglobin

17th Aug, 2026

10.8 g/dL

OUT OF RANGE

- The Hemoglobin test measures the oxygen-carrying protein in red blood cells, essential for oxygen transport.
- Low levels reduce oxygen to the gut, causing weak digestion and infection risk. High levels may affect blood flow, leading to digestive issues.

Reference range

Your result (10.8 g/dL) is **out of range low**

Inference	Value (g/dL)
Critical Low	<=6
Out of range Low	6-11
Borderline Low	11-12
Ideal (Normal Range)	12-16.5



RDW-CV

17th Aug, 2026

15.8 %

BORDERLINE

- This test calculates the percentage variation in red blood cell size, helping detect nutrient deficiencies or blood disorders.
- A slightly high RDW-CV means your red cells are not all the same size. This may happen if your body is adjusting after low iron, folate, or vitamin B12. If hemoglobin or ferritin are also low, that supports it. Often not urgent, but repeating the test is helpful.

Reference range

Your result (15.8 %) is **borderline high**

Inference	Value (%)
Critical High	>=16.1
Out of range High	16-16.1
Borderline High	14.5-16
Ideal (Normal Range)	11.5-14.5

**MCV**

17th Aug, 2026

81.6 fL**BORDERLINE**

- This test measures the average size of your red blood cells.
- A slightly low MCV means red cells are a bit smaller. This may follow iron shortage or poor diet. If Hb or ferritin are also low, that supports it. Worth repeating, as it often improves with time.



Triglycerides

17th Aug, 2026

297 mg/dL

OUT OF RANGE

- This test measures the levels of triglycerides, a type of fat in the blood. High levels may increase the risk of heart disease.
- Low triglycerides mean less fat is circulating in the blood than usual. This can follow diet changes, weight loss, or long fasting. If cholesterol is also low, it adds clues. Not usually urgent, but review helps guide care.

Reference range

Your result (297 mg/dL) is **out of range high**

Inference	Value (mg/dL)
Out of range High	>199
Borderline High	150-199
Ideal (Normal Range)	0-150



PCV

17th Aug, 2026

33.80 %

BORDERLINE

- The PCV test shows what percentage of your blood is made up of red blood cells, which carry oxygen.
- A slightly low PCV means the proportion of red cells is a bit reduced. This may follow low iron foods, recent illness, or fluid overload. If Hb or MCV are also low, that supports it. Worth repeating to track.

Reference range

Your result (33.80 %) is **borderline low**

Inference	Value (%)
Critical Low	<=18
Out of range Low	18-32.4
Borderline Low	32.4-36
Ideal (Normal Range)	36-50



General recommendations

- Exercise regularly; do strength exercises (e.g. weight lifting) and stretching exercises (e.g. yoga) 2-3 times a week.
- Try getting at least 150 minutes of exercise each week and don't go over 2 days in a row without some form of physical activity.
- Opt for a healthy meal plan consisting of fresh fruits and vegetables, lean protein, low-fat dairy products, complex carbohydrates, and healthy fats.
- Manage stress and maintain a healthy weight.
- Check your feet and skin regularly for redness, cuts, bruises, blisters and sores.